

Indigestion

(Dyspepsia)

By **Jonathan Gotfried**, MD, Main Line Health, Bryn Mawr, PA

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Indigestion is pain or discomfort in the upper abdomen. People may also describe the sensation as gassiness, a sense of fullness, or gnawing or burning. The sense of fullness may occur after a small meal (early satiety), be a feeling of excessive fullness after a normal-sized meal (postprandial fullness), or be unrelated to meals.

Because dyspepsia is usually a vague, mild discomfort, many people do not seek medical care until it has been present (or coming and going) for a long time. Sometimes, dyspepsia is a more sudden, noticeable (acute) sensation.

Depending on the cause of the dyspepsia, people may have other symptoms such as a poor appetite, nausea, constipation, diarrhea, flatulence, and belching. For some people, eating makes symptoms worse. For others, eating relieves symptoms.

Causes of Indigestion

Dyspepsia has many causes, which, despite common use of the term "indigestion," do not involve a problem digesting food.

Acute dyspepsia may occur briefly after ingestion of:

- A large meal
- Alcohol
- Certain irritating medications (such as bisphosphonates, erythromycin, iron, or nonsteroidal anti-inflammatory drugs [NSAIDs])

Also, some people having a heart attack or unstable angina (coronary artery ischemia) may feel only a sensation of dyspepsia, rather than chest pain (see [Chest or Back Pain](#)).

For **recurring dyspepsia**, common causes include:

- Cancer (of the [stomach](#) or [esophagus](#))
- Delayed gastric (stomach) emptying
- Medications
- [Gastroesophageal reflux disease \(GERD\)](#)
- [Gastritis](#) or [peptic ulcer disease](#)
- [Helicobacter pylori infection](#)
- Unknown (nonulcer dyspepsia)

Delayed gastric emptying is a situation in which food remains in the stomach for an abnormally long period of time. Delayed emptying is usually caused by a disorder (such as diabetes, a connective tissue disorder, or a neurologic disorder) that affects the nerves to the digestive tract.

Anxiety by itself does not cause dyspepsia. However, anxiety can sometimes worsen dyspepsia by increasing the person's concern about unusual or unpleasant sensations, so that minor discomfort becomes very distressing.

In many people, doctors find no abnormality during a physical examination or after looking in the esophagus and stomach with a flexible viewing tube (upper endoscopy) or after doing imaging or laboratory tests. In such cases, called nonulcer dyspepsia (functional dyspepsia), the person's symptoms may be due to an increased sensitivity to sensations in the stomach or to intestinal contractions.

Evaluation of Indigestion

Not every episode of dyspepsia requires immediate evaluation by a doctor. The following information can help people decide when a doctor's evaluation is needed and help them know what to expect during the evaluation.

Warning signs

In people with dyspepsia, certain symptoms and characteristics are cause for concern. They include

- Shortness of breath, sweating, or fast heart rate accompanying an episode of dyspepsia
- [Loss of appetite](#) (anorexia)
- [Nausea or vomiting](#)

- Weight loss
- Blood in the stool
- [Difficulty swallowing](#) (dysphagia) or pain with swallowing (odynophagia)
- Dyspepsia that persists despite treatment with medications such as proton pump inhibitors (PPIs)

When to see a doctor

People who have a single, sudden episode of dyspepsia should see a doctor right away, especially if their symptoms are accompanied by shortness of breath, sweating, or a fast heart rate. Such people may have [acute coronary ischemia](#). People with chronic dyspepsia that occurs when they exert themselves but that goes away when they rest may have [angina](#) and should see a doctor within a few days.

People with dyspepsia and one or more of the other warning signs should see a doctor within a few days to a week. Those with recurrent dyspepsia and no warning signs should see a doctor at some point, but a delay of a week or so is not harmful.

What the doctor does

Doctors first ask questions about the person's symptoms and medical history. Doctors then do a physical examination. What they find during the history and physical examination often suggests a cause of the dyspepsia and the tests that may need to be done (see table [Some Causes and Features of Indigestion](#)).

The history is focused on obtaining a clear description of the symptoms, including whether they are sudden or chronic. Doctors need to know the timing and frequency of recurrence, any difficulty swallowing, and whether the symptoms occur only after eating, drinking alcohol, or taking certain medications or illicit drugs. Doctors also need to know what factors make the symptoms worse (particularly exertion, certain foods, or alcohol) or relieve them (particularly eating or taking antacids).

Doctors also ask the person about gastrointestinal symptoms such as loss of appetite, nausea, vomiting, vomiting of blood (hematemesis), weight loss, and bloody or black stools. Other symptoms include shortness of breath and sweating.

Doctors need to know whether the person has been diagnosed with a gastrointestinal and/or heart disorder, has any heart risk factors (such as high blood pressure [\[hypertension\]](#) or an excessive amount of cholesterol in the blood [\[hypercholesterolemia\]](#)), and the results of previous tests that have been done and treatments that have been tried.

The physical examination usually does not give doctors clues to a specific diagnosis. However, doctors look for signs of chronic disease, such as very pale skin, wasting away of muscle or fat tissue (cachexia), or yellowing of the eyes and skin ([jaundice](#)). They also do a rectal examination to detect any blood. Doctors are more likely to recommend testing for people who have any of these abnormal findings.



Some Causes and Features of Indigestion

Cause	Common Features*	Tests
Cancer (such as cancer of the esophagus or stomach cancer)	Chronic, vague discomfort Later, dysphagia with esophageal cancer or early satiety with stomach cancer Weight loss	Upper digestive tract endoscopy (examination of the esophagus, stomach, and duodenum using a flexible viewing tube called an endoscope) CT of the abdomen
Celiac disease	Change in bowel habits, such as diarrhea, bloating, or greasy stools Iron deficiency anemia that has no other cause	Blood tests Endoscopic biopsy of tissue from the small intestine
Coronary ischemia (inadequate blood flow to the coronary arteries)	Sometimes in people who have symptoms when exerting themselves Risk factors for heart disorders (such as high blood pressure, diabetes, and/or high cholesterol levels)	Electrocardiography (ECG) Blood tests Sometimes stress testing
Medications (such as bisphosphonates , erythromycin and other macrolide antibiotics, estrogens, iron, nonsteroidal anti-inflammatory drugs [NSAIDs], and MSD Pharmaceuticals , Merck & Co., Inc. , Rahway, NJ, USA and its affiliates. All rights reserved.	In people who are taking a medication that can cause indigestion Symptoms occur shortly after taking the medication	A doctor's examination